



Where Should She Be Managed?

A field guide to the high-risk referral ladder for pregnant and postnatal women



BEmOC



CEmONC (SDH)



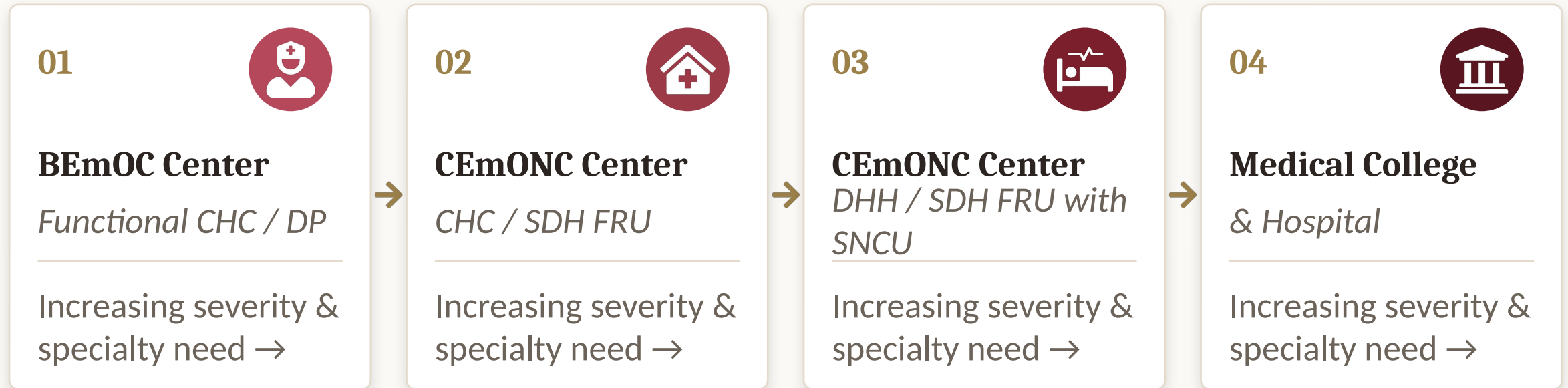
CEmONC
(DHH/SNCU)



Medical
College

The Referral Ladder

Four facility levels, increasing capability — match the woman's risk condition to the right level



How to use this SOP: Each level below lists the high-risk conditions it is meant to manage, and who is authorised to refer the woman onward (ANM, treating physician, or both). Always refer upward without delay once a listed condition is identified — do not wait for the condition to worsen.



LEVEL 1 OF 4

BEmOC Center

Functional CHC / DP



- History of pre-eclampsia / eclampsia / PIH convulsion during previous pregnancy
- History of active TB / leprosy
- Pregnancy Induced Hypertension (BP > 140/90) without proteinuria
- Presence of sugar in urine
- Premature rupture of membrane / leaking
- History of STI / RTI / HIV positive
- Pregnancy with HIV positive / STI / RTI



WHO REFERS

- ▶ ANM
- ▶ Treating Physician
- ▶ (considering type of services required)



LEVEL 2 OF 4

CEmONC Center

CHC / SDH FRU



- More than 4 earlier pregnancies
- Previous Caesarean Section / any uterine surgery
- Foul smelling vaginal discharge
- Bleeding PV
- History of previous complicated delivery
- History of repeated spontaneous abortion (more than 3 times)
- Premature contraction with pregnancy > 32 weeks



WHO REFERS

- ▶ Treating Physician of BEmOC, if complication arises
- ▶ ANMs may directly refer clinically high-risk Red Card holder mothers to CEmONC (considering type of services required)



LEVEL 3 OF 4

CEmONC Center

DHH / SDH FRU having SNCU



- Previous LBW baby delivery / pre-term delivery / Intra Uterine Growth Retardation (IUGR) baby
- Previous neonatal death / still birth / birth asphyxia
- Weight gain <500gm/month from 20 weeks onward, or >3kg/month at any time during pregnancy
- Pregnancy Induced Hypertension (BP >140/90) with proteinuria
- Pregnant woman with malaria / any high fever
- Diagnosed multiple pregnancy (twin / triplet)
- Over-distended / unusually large size of uterus
- Decreased / no fetal movement
- History of APH / PPH
- Height < 140 cm

- History of diabetes in pregnancy or GDM
- 1st pregnancy at age < 18 years
- Any pregnancy at age above 35 years
- Severe anemia (< 7 gm/dl)
- Hepatitis B positive / jaundice
- Ectopic pregnancy and rupture uterus
- Premature contraction with pregnancy < 32 weeks
- History of Thalassemia / Sickle Cell Disease / Rh-negative mother (non-isoimmunised)
- Post-dated pregnancy (< 7 days from EDD)
- Conceived after long period of infertility / artificial reproductive techniques



WHO REFERS:

Treating Physician of BEmOC / CEmONC (CHC/SDH), if complication arises
holder mothers (considering type of services required)

► ANMs may directly refer the Clinically High-Risk Red Card



LEVEL 4 OF 4

Medical College & Hospital

Highest level — super-specialty care



- History of heart disease
- History of liver disease
- History of kidney disease
- Rh-negative mother (isoimmunised)
- Post-dated pregnancy (more than 7 days from EDD)
- Diagnosed placenta accreta
- Caesarean scar pregnancy
- Previous caesarean with placenta previa
- Case of DIC (Disseminated Intravascular Coagulation)
- Any critical patient in antenatal, intranatal or postnatal period requiring super-specialty attention



WHO REFERS

- ▶ Treating Physician of BEmOC / CEmONC, if complication arises
- ▶ ANMs may directly refer the Clinically High-Risk Red Card holder mothers (considering type of services required)

Quick Recall: Match the Risk to the Level

Use this as a field cheat-sheet — one defining example per level

	BEmOC	BP >140/90 without proteinuria, leaking PV, HIV/STI/RTI positive
	CEmONC — SDH	Previous LSCS, bleeding PV, >4 prior pregnancies, repeated abortions
	CEmONC — DHH/SNCU	Severe anemia, twin pregnancy, GDM, age <18 or >35, prior stillbirth
	Medical College	Heart / liver / kidney disease, placenta accreta, DIC, any critical case

Full criteria and complete condition lists are listed in the preceding level slides and the original Referral SOP (Annexure A).